

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Bexsero for vaccination against Meningococcal Group B

Summary of NICE / NICE CKS Guidelines for Meningitis B Vaccination — Background Reference Only

Overview

Note: The summary below describes the NHS national immunisation programme for contextual reference. This PGD operates as a private vaccination service for individuals aged 2 years and older who have not previously been immunised via the NHS programme, or who require vaccination for occupational health or travel purposes. Eligible infants should be referred to the NHS national immunisation service. The NHS infant schedule, prophylactic paracetamol guidance for infants, and catch-up schedules described below do not apply to this private service.

- **Meningococcal group B (MenB)** is a leading cause of **bacterial meningitis and septicaemia**, particularly in **infants and young children**.
- The **MenB vaccine (Bexsero®)** is a **protein-based vaccine** that protects against invasive **Neisseria meningitidis group B** disease.

Routine Childhood Immunisation

Schedule (UK NHS programme):

- **First dose:** at **8 weeks** of age
- **Second dose:** at **16 weeks**
- **Booster:** at **12–13 months** (toddler booster)

Included in the **routine UK childhood immunisation schedule** since **September 2015**.

High-Risk Groups

MenB vaccine is also recommended for:

- Individuals with **functional or anatomical asplenia**

- Those with **complement deficiencies** or on **complement inhibitor therapy** (e.g. eculizumab)
 - Close contacts of a **confirmed MenB case**
 - During **outbreak control** in specific settings
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Catch-Up Vaccination

- Available for certain age groups or clinical risk groups on specialist advice.
 - No routine catch-up for children born before the introduction of the programme (pre-2015), unless in high-risk categories.
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Administration

- **Intramuscular injection** into the deltoid muscle (individuals aged 2 years and older). The anterolateral thigh may be used where the deltoid is not accessible.
 - Can be co-administered with other childhood vaccines, but **increased risk of fever**.
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Prophylactic Paracetamol

- Recommended for **infants under 1 year** receiving MenB vaccine, especially at **8 and 16 weeks**:
 - First dose **as soon as possible** after vaccination.
 - Two further doses at **4–6 hour intervals**.
 - Reduces risk of **post-vaccination fever**.
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Contraindications

- Severe allergic reaction to a previous dose or a vaccine component.
 - Defer in the case of acute severe febrile illness (minor infections are not a reason to delay).
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Side Effects

- Common: Fever (especially when given with other vaccines), injection site pain/swelling, irritability, loss of appetite.
- Rare: Hypersensitivity reactions.

Patient Group Direction

for the supply/administration of

Bexsero

for vaccination against

Meningococcal Group B

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training with Get Real Health in order to use this PGD. This training will be documented and signed off by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Bexsero is indicated for active immunisation against invasive disease caused by <i>Neisseria meningitidis</i> group B in individuals from 2 years of age and older, where not previously immunised via the national NHS vaccination programme, or where vaccination is required for occupational health or travel purposes.
Inclusion criteria	- Individuals 2 years of age and older. - Those who have not received immunisation via the national NHS vaccination programme. - Those who require vaccination for occupational health reasons. - Those who request the vaccine for travel purposes. - Informed consent obtained. - No contraindications to vaccination.
Exclusion criteria	- Known hypersensitivity to the active substance or any excipients. - Previous severe allergic reaction to a meningococcal B vaccine. - Acute febrile illness (vaccination should be postponed until recovered). - Individuals with sensitivity to Kanamycin (kanamycin is used in the early manufacturing process and is removed during later stages of manufacture; if present, levels in the final vaccine are less than 0.01 micrograms per dose, but safe use in kanamycin-sensitive individuals has not been established). - Pregnancy: the potential risk for pregnant women is unknown. Vaccination should not be withheld when there is a clear risk of exposure to meningococcal infection, but such individuals should be referred to their GP. - Breast-feeding: information on the safety of the vaccine to women and their children during breast-feeding is not available. The benefit-risk ratio must be examined before making the decision to immunise during breast-feeding and such individuals should be referred to their GP.
Cautions	- Paracetamol may be recommended post-vaccination to reduce risk of fever, particularly in younger children. - Observe for 15 minutes post-vaccination. - Administer in accordance with age-specific schedule and spacing intervals. - Inform recipient/guardian of common side effects and follow-up requirements. - There is no data on the use of Bexsero in subjects above 50 years of age; use with caution and refer to GP or specialist clinician. - There is limited data on the use of Bexsero in patients with chronic medical conditions; use with caution and refer to GP or specialist clinician. - Individuals with an impaired immune response may have a reduced antibody response to the vaccine. - Individuals with familial complement deficiencies (for example, C3 or

	C5 deficiencies) and individuals receiving treatments that inhibit terminal complement activation (for example, eculizumab) are at increased risk for invasive disease caused by <i>Neisseria meningitidis</i> group B, even if they develop antibodies following vaccination with Bexsero. - As with all injectable vaccines, appropriate medical treatment and supervision should always be readily available in case of an anaphylactic event following administration of the vaccine. Anxiety-related reactions may also occur in association with vaccination; procedures must be in place to avoid injury from fainting.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Bexsero Meningococcal Group B vaccine for injection
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Intramuscular injection into the deltoid muscle. The anterolateral thigh may be used in younger children aged 2–3 years where the deltoid muscle mass is insufficient. This vaccine should not be given to individuals with thrombocytopenia or any coagulation disorder that would contraindicate intramuscular injection, unless the potential benefit clearly outweighs the risk of administration. The vaccine must not be injected intravenously, subcutaneously or intradermally and must not be mixed with other vaccines in the same syringe. Separate injection sites must be used if more than one vaccine is administered at the same time.
Dose and frequency	Two doses of 0.5 mL not less than 1 month apart. A booster should be considered in individuals who are immunosuppressed or who are at continued risk of exposure to meningococcal disease.
Quantity to be administered and/or supplied	0.5 mL per dose.
Maximum or minimum treatment period	To complete the course and offer protection, two doses of 0.5 mL should be administered not less than 1 month apart.
Adverse effects	Fever, irritability, tenderness at injection site, sleepiness, loss of appetite. Older individuals: headache, nausea, muscle pain. Rare: allergic reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	- Inform the individual/parent/carer of possible side effects and how they should be managed. - Inform the individual/parent/carer that the vaccine will not protect against other strains of the meningococcal group. - Individuals should be advised to seek medical advice in the event of a severe adverse reaction. - Individuals should be warned that no vaccine is 100% effective. In the case of systemic upset, acute illness or any other clinical concerns, the patient should be referred urgently for medical assessment. - Advise the individual/parent/carer of preventative measures to reduce exposure (i.e. avoid crowded areas, hand and respiratory hygiene).

Key references

● NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mpg2 ● NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources ● NICE CKS Guidance ● British National Formulary (BNF) Key information on the selection, prescribing, dispensing and

administration of medicines [BNF \(British National Formulary\)](#) | [NICE](#)

● Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		17/3/2026
Chris Pilkington	Pharmacist GPhC: 2046322		17/3/2026

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025
002	Updated in the light of recent outbreak	17 th March 2026